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Saveetha Institute of Medical and Technical Sciences

Chennai – 602105

MEDIGUARD AI

**An Intelligent Digital Prescription and Medication Adherence
Management System**

A CAPSTONE PROJECT REPORT

Submitted by

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Computer science and engineering

Under the Supervision of

Dr. Latha

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BONAFIDE CERTIFICATE

This is to certify that the Capstone Project entitled “MediGuard AI: An Intelligent Digital Prescription and Medication Adherence Management System” has been carried out by S Jefna Princy(192572058) Department of Computer Science and Engineering, under the supervision of Dr. Latha, and is submitted in partial fulfilment of the requirements for the course of the Bachelor of Engineering at SIMATS Engineering, Saveetha Institute of Medical and Technical Sciences, Chennai.

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DECLARATION

I, S Jefna Princy, of the Department of Computer Science and Engineering, SIMATS Engineering, Saveetha Institute of Medical and Technical Sciences, hereby declare that the Capstone Project Work entitled “MediGuard AI: An Intelligent Digital Prescription and Medication Adherence Management System” and this individual Module 3 report are the result of my academic work. The report presents the Secure Prescription Verification & Healthcare Access module based on the uploaded project source material. All sources used have been acknowledged, and no unsupported numerical performance result has been presented as a completed project finding.

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INDIVIDUAL CONTRIBUTION STATEMENT

This statement records the individual contribution documented in this report. The available project source identifies Module 3 as Secure Prescription Verification & Healthcare Access. Detailed implementation metrics and code were not included in the supplied presentation; therefore, the contribution description is limited to the module scope documented by the source.

Student Name / Register No.	Specific Responsibilities	Design & Development Contribution	Testing & Analysis Contribution	Report Contribution	Approx. Contribution (%)
DHARSHINI R D 192521277	Study and document Module 3 requirements	Document secure prescription verification, QR-code authentication, access control, encryption, and authorized sharing	Describe verification objectives and security considerations; quantitative test results not supplied	Preparation of Module 3 report	33%
JEFNA PRINCY S 192572058	Study and document Module 1 requirement	Designed the user interface, patient registration, prescription input, and medicine database modules. Implemented user-friendly navigation and data storage functionality.	Performed functional testing of registration, prescription entry, and medicine-record modules. Verified data accuracy and system usability.	Performed functional testing of registration, prescription entry, and medicine-record modules. Verified data accuracy and system usability.	33%
RAKSHITHA 192411008	Study and document Module 2 requirement	Developed medication adherence monitoring, reminder/notification, dosage tracking, and patient medication-history features.	Tested medication reminders, adherence tracking, dosage information, and notification functionality. Analyzed test results and identified errors.	Preparation of Module 2 report, including testing results, screenshots, and module documentation.	34%
TOTAL					100%

ABSTRACT

Prescription management is an important part of healthcare, as incorrect, lost, forged, or misinterpreted prescriptions can contribute to medication errors and affect patient safety. Traditional paper-based prescription systems make it difficult to maintain, access, and manage prescription information efficiently. Therefore, there is a need for a centralized digital solution that can simplify prescription management and improve the reliability and accessibility of prescription records.

This project proposes MediGuard AI: An Intelligent Digital Prescription and Medication Adherence Management System, with Module 1 – Intelligent Digital Prescription Management focusing on the digital management of prescription information. The proposed system provides a centralized platform for managing prescriptions and enables authorized users such as doctors, patients, and pharmacists to access prescription-related information through a digital environment. The system aims to reduce problems associated with paper-based prescriptions, including loss, misinterpretation, and unauthorized modification.

The methodology involves converting prescription management into a structured digital workflow. Prescription information is digitally created and maintained so that relevant details can be accessed through the system when required. The platform is designed with a focus on secure handling of prescription information and improved communication between healthcare participants. By maintaining prescription data digitally, the system supports easier retrieval and organized management compared with traditional paper-based methods.

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LIST OF ABBREVIATIONS / SYMBOLS

Symbol / Abbreviation	Description	Unit
AI	Artificial Intelligence	—
QR	Quick Response code	—
MediGuard AI	Project system name	—
API	Application Programming Interface	—
ID	Identity / user identifier	—

CHAPTER 1 – INTRODUCTION

1.1 Background

Prescription management is a fundamental activity in healthcare, as accurate prescription information is required for safe and effective medication use. Traditionally, prescriptions are commonly maintained using paper-based methods. Although this approach is familiar and simple, it can create difficulties in storing, retrieving, reading, and managing prescription information. Paper prescriptions may also be lost, damaged, misinterpreted, or subject to unauthorized changes, which can contribute to medication-related errors.

The MediGuard AI: An Intelligent Digital Prescription and Medication Adherence Management System is proposed as a digital healthcare platform that brings prescription-related activities into a centralized environment. The system connects doctors, patients, and pharmacists through a digital platform and is intended to improve the efficiency and reliability of prescription management. The overall project combines digital prescriptions, medication management, and secure healthcare functions, while Module 1 specifically focuses on Intelligent Digital Prescription Management.

Module 1 provides the foundation of the system by enabling prescription information to be digitally created, organized, stored, and accessed. Instead of depending completely on physical documents, prescription information can be maintained in a structured digital format. This provides a more convenient approach to prescription management and supports better accessibility of healthcare information.

1.2 Need for the Project

The problem of inefficient prescription management needs to be addressed because prescription information directly affects medication usage and patient safety. Paper-based prescriptions can be lost, damaged, misunderstood, or difficult to retrieve when required. Such limitations can increase the possibility of prescription-related errors and make healthcare information management less efficient.

The major stakeholders affected include doctors, patients, pharmacists, and healthcare providers. Doctors require an organized method to manage prescriptions, patients need reliable access to their prescription information, and pharmacists need clear prescription details for appropriate medication dispensing.

Existing paper-based approaches have several limitations:

- Dependence on physical documents for storing prescription information.
- Risk of loss or damage of prescriptions.
- Difficulty in retrieving older prescription records.
- Possibility of misinterpretation due to unclear handwritten information.
- Lack of a centralized digital platform for prescription management.
- Increased possibility of errors during manual handling.

From an engineering perspective, the project is significant because it requires the design of a digital system that balances accessibility, organization, reliability, security, and usability. The proposed solution aims to provide a structured digital environment for prescription management while reducing the limitations of conventional paper-based methods. The identified need for a secure and centralized prescription-management system is consistent with the project's stated problem definition.

1.3 Problem Statement

The engineering problem is to design and develop a centralized intelligent digital prescription management module that can organize and manage prescription information while addressing the limitations of conventional paper-based prescription handling.

The system must manage multiple interacting requirements, including accurate prescription data entry, structured storage, easy retrieval, controlled access, usability, and reliable information handling. At the same time, the system must reduce the risks associated with lost, damaged, or misinterpreted prescriptions without making the process unnecessarily complex for healthcare users.

The engineering challenge therefore involves developing a practical digital solution that provides:

- **Accurate and organized prescription information.**
- **Efficient digital storage and retrieval.**
- **Convenient access for authorized healthcare participants.**
- **Reliable handling of prescription records.**
- **Improved usability compared with paper-based methods.**
- **Appropriate protection of prescription information.**

1.4 Project Aim

The aim of Module 1 – Intelligent Digital Prescription Management is to design and develop a centralized digital system for creating, organizing, storing, and accessing prescription information, thereby improving prescription-management efficiency, accessibility, and reliability.

1.5 Project Objectives

- 1. To design a structured digital framework for prescription management.**
- 2. To develop a centralized mechanism for creating and maintaining digital prescription records.**
- 3. To organize prescription information in a clear and accessible format.**
- 4. To implement efficient storage and retrieval of prescription records.**
- 5. To provide appropriate access to prescription information for relevant healthcare users.**
- 6. To test the prescription-management workflow for accuracy, usability, and reliable data handling.**
- 7. To evaluate the proposed digital approach against the limitations of conventional paper-based prescription management.**

1.6 Scope

Included

The scope of Module 1 includes:

- **Digital creation and management of prescription records.**
- **Structured storage of prescription information.**
- **Organization of medicine and prescription details.**
- **Retrieval and viewing of stored prescription information.**
- **Centralized management of prescription records.**
- **User-oriented access to relevant prescription information.**
- **Testing of the digital prescription-management workflow.**

Excluded

The following functions are outside the primary scope of Module 1:

- **Medication reminders and adherence monitoring.**
- **Missed-dose tracking and adherence reports.**
- **QR-code-based prescription verification.**
- **Broader healthcare-access and authentication functions.**

These functions belong to the other modules of the overall MediGuard AI system. The project structure identifies Module 2 as medication adherence and Module 3 as secure prescription verification and healthcare access.

Assumptions

- **Users provide correct prescription information during digital entry.**
- **Authorized users have appropriate access to the system.**
- **The system has a functioning digital storage mechanism.**
- **Required prescription information is available for entry and management.**
- **Users have basic familiarity with digital healthcare applications.**

Boundary Conditions

The module focuses specifically on prescription information management and does not replace professional medical decision-making. The system manages and presents prescription information but does not independently diagnose diseases or determine treatment plans.

1.7 Expected Engineering Outcome

The expected engineering outcome is a functional prototype of an Intelligent Digital Prescription Management Module within the MediGuard AI system.

The prototype is expected to provide a structured digital workflow through which prescription information can be created, stored, organized, retrieved, and accessed by appropriate users. It should demonstrate how digital prescription management can reduce dependence on paper-based records and improve the accessibility and organization of prescription information.

The expected outcome includes:

- A centralized digital prescription-management system.
- Structured prescription records.
- Efficient prescription storage and retrieval.
- Improved accessibility of prescription information.
- A user-friendly prescription-management workflow.
- Reliable handling of digital prescription data.

Overall, Module 1 is expected to establish the core prescription-management foundation of MediGuard AI and provide a more organized and efficient alternative to conventional paper-based prescription handling.

CHAPTER 2 – PROJECT BACKGROUND AND EXISTING APPROACH

2.1 Review Method

The literature review was conducted to identify existing approaches, technologies, and systems related to digital prescription management and electronic healthcare records. The review focused on research and technological solutions that address the creation, storage, organization, retrieval, and management of prescription information.

Relevant sources were identified by examining research publications, healthcare technology solutions, and established digital prescription approaches. The review considered the advantages, limitations, technical requirements, and practical applicability of existing solutions. Particular attention was given to how existing approaches address the limitations of paper-based prescriptions, including information loss, difficulty in retrieval, and misinterpretation.

The review was used to identify the engineering requirements for Module 1 and determine the limitations that could be addressed through the proposed Intelligent Digital Prescription Management module.

2.2 Review of Existing Work

Digital Prescription Systems

Electronic prescribing systems have been developed to replace or reduce dependence on handwritten prescriptions. These systems allow prescription information to be entered and stored electronically, improving readability and reducing the possibility of errors caused by unclear handwriting. Digital prescriptions can also be accessed more easily than physical documents.

However, electronic prescribing solutions can vary significantly in their functionality and integration capabilities. Some systems are designed primarily for prescription generation, while others are integrated into larger hospital or electronic health-record platforms. Complex systems may also require extensive infrastructure and user training.

Electronic Health Record Systems

Electronic Health Record (EHR) systems provide centralized storage and management of patient healthcare information. Prescription information can form part of a patient's electronic medical record, allowing healthcare professionals to access historical information when required.

The major advantage of EHR-based approaches is centralized information management. However, comprehensive EHR platforms can be technically complex because they manage large amounts of healthcare data beyond prescriptions. Such complexity may make them less suitable when the primary requirement is a focused prescription-management solution.

Digital Healthcare Platforms

Modern healthcare platforms increasingly use web and cloud-based technologies to provide centralized access to healthcare information. These approaches improve accessibility and reduce dependence on physical documents. Digital platforms can also provide structured data storage and allow information to be retrieved more efficiently.

A major limitation is that healthcare applications must balance accessibility with privacy and controlled data access. Prescription information is sensitive healthcare information, making secure data handling an important engineering requirement.

AI-Assisted Healthcare Systems

Artificial Intelligence is increasingly being incorporated into healthcare systems to support information processing and decision-making. In a prescription-management context, AI can potentially assist in organizing prescription information and supporting intelligent handling of digital records.

However, AI should be applied carefully in healthcare because incorrect automated interpretation or recommendations can introduce risks. Therefore, the proposed Module 1 focuses primarily on intelligent digital management of prescription information rather than replacing medical professionals in prescribing or treatment decisions.

Overall Review

2.3 Comparative Analysis

Existing Approach	Advantages	Limitations	Relevance to Proposed Work
Paper-based prescriptions	Simple, familiar, does not require digital infrastructure	Can be lost, damaged, misread, or difficult to retrieve	Provides the main problem that Module 1 aims to address
Electronic prescribing systems	Improves readability and reduces paper dependency	May require specific infrastructure and integration	Provides the basic foundation for digital prescription creation
Electronic Health Records (EHR)	Centralized patient information and historical records	Large and complex systems; prescription management is only one component	Inspires centralized storage and retrieval of prescription records
Web-based healthcare systems	Convenient access and centralized information management	Requires reliable connectivity and appropriate access control	Supports the use of a centralized digital platform
Cloud-based healthcare solutions	Scalable storage and accessibility across locations	Security, privacy, and infrastructure dependency must be considered	Provides a possible approach for storing digital prescription records
AI-assisted healthcare systems	Can support intelligent information processing and organization	Requires careful validation and should not replace professional medical judgment	Provides the intelligent aspect of the proposed prescription-management module

Existing Approach	Advantages	Limitations	Relevance to Proposed Work
Integrated healthcare platforms	Can connect multiple healthcare activities in one environment	Can be costly and technically complex	Demonstrates the value of connecting prescription information within a unified system
Proposed MediGuard AI – Module 1	Focused prescription management, structured digital records, centralized handling, improved accessibility	Requires appropriate software infrastructure, secure data management, and user adoption	Directly addresses the identified limitations of paper-based prescription management

2.4 Research / Engineering Gap

The literature and existing solutions indicate that digital prescription technologies can overcome several limitations of paper-based prescriptions. However, an engineering gap remains in developing a focused, practical, and user-oriented prescription-management module that combines structured digital prescription handling with centralized storage and easy retrieval.

Large healthcare information systems provide extensive functionality but can introduce unnecessary complexity when the primary requirement is prescription management. Conversely, simpler digital prescription approaches may focus mainly on prescription generation without providing a broader structured management workflow.

Another important gap is balancing multiple requirements simultaneously. A practical system must provide convenient access while maintaining reliable information handling, appropriate security, usability, and manageable technical complexity.

Therefore, the unresolved engineering requirement is to develop a prescription-management solution that is:

- Centralized but not unnecessarily complex.
- Digital and easily accessible.
- Structured for efficient prescription management.
- Reliable in storing and retrieving prescription information.
- Simple enough for practical user interaction.
- Designed with healthcare-data security considerations.
- Capable of providing intelligent support without replacing professional medical decisions.

2.5 Proposed Contribution

The proposed MediGuard AI – Module 1: Intelligent Digital Prescription Management addresses the identified engineering gap by developing a focused digital environment for prescription management.

The proposed module contributes a structured workflow for creating, organizing, storing, and retrieving prescription information in a centralized system. Instead of depending on physical prescription documents, the system maintains prescription information in digital form, improving accessibility and organization.

The key contribution of Module 1 is its focus on prescription management as a dedicated engineering problem rather than attempting to replace a complete hospital information system. The design aims to achieve an appropriate balance between functionality, usability, accessibility, reliability, and technical complexity.

The proposed contribution can therefore be summarized as:

1. Digital transformation – Converts conventional prescription handling into a structured digital process.
2. Centralized management – Provides a common environment for maintaining prescription information.

- 3. Improved accessibility – Makes stored prescription information easier to retrieve when required.**
- 4. Structured information handling – Organizes prescription details in a consistent digital format.**
- 5. Reduced paper dependency – Minimizes problems associated with physical prescription storage.**
- 6. Intelligent management approach – Incorporates intelligence at the system-management level while keeping medical decision-making with qualified healthcare professionals.**

Thus, Module 1 forms the core prescription-management component of MediGuard AI and provides the digital foundation upon which the other modules of the overall system can operate.

CHAPTER 3 – MODULE 3 DESIGN AND SECURITY CONSIDERATIONS

3.1 Engineering Requirements

The engineering requirements for Module 1 – Intelligent Digital Prescription Management are derived from the limitations of traditional paper-based prescription handling and the need for an organized digital solution.

The system should satisfy the following requirements:

Functional Requirements

- The system shall allow authorized users to create digital prescription records.
- The system shall store prescription information in a structured format.
- The system shall allow prescription records to be retrieved when required.
- The system shall display prescription information in a clear and understandable manner.
- The system shall maintain organized records for efficient prescription management.
- The system shall support centralized handling of prescription information.

Non-Functional Requirements

- **Usability:** The interface should be simple and easy to understand.
- **Reliability:** Prescription information should be stored and retrieved consistently.
- **Security:** Prescription data should be protected from unauthorized access.
- **Performance:** Common prescription-management operations should be completed efficiently.
- **Maintainability:** The system should be easy to update and maintain.
- **Scalability:** The design should allow additional prescription records to be handled as system usage increases.

3.2 Constraints and Applicable Considerations

The development of the module is subject to several practical engineering constraints.

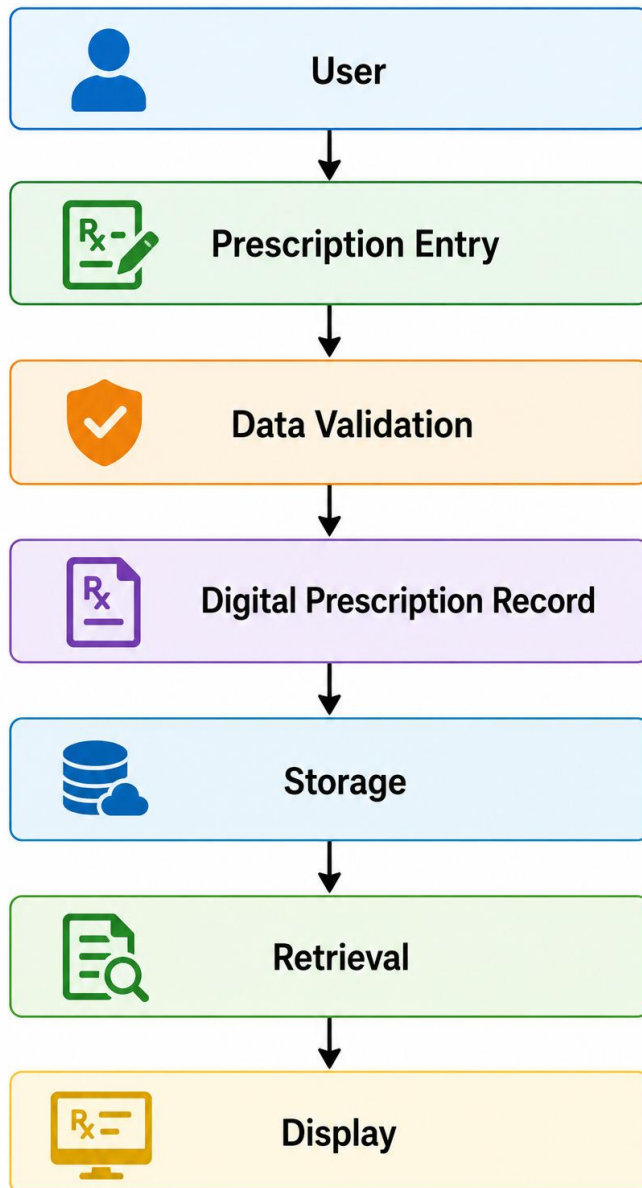
Constraint	Consideration
Data privacy	Prescription information must be handled carefully because it contains healthcare-related data
System resources	The prototype should operate using available computing resources
Usability	The interface should not make prescription management unnecessarily complicated
Data accuracy	Incorrectly entered prescription information can affect the usefulness of the system
Connectivity	Digital access may depend on the availability of the underlying system infrastructure
Security	Access to prescription information must be appropriately controlled
Technical complexity	The system should provide useful functionality without unnecessary complexity

The project also considers established digital-health practices and the importance of responsible management of healthcare information. The overall project references WHO digital-health guidance as part of its technical context.

3.3 Design Specifications

The proposed Module 1 is designed around a centralized prescription-management workflow.

The basic workflow is:



The prescription record is organized so that relevant information can be maintained consistently. Depending on the system design, the record can contain details such as:

- Patient identification information
- Doctor/prescriber information
- Prescription date
- Medicine name
- Dosage information
- Frequency and duration
- Additional prescription instructions

The system should separate different categories of information rather than storing the entire prescription as unstructured text. This improves organization and makes future retrieval and management easier.

3.4 Alternative Solutions and Evaluation

Three broad approaches were considered for managing prescription information.

Approach	Advantages	Limitations
Paper-based system	Simple and familiar	Risk of loss, damage and misinterpretation; difficult retrieval
Basic digital prescription system	Reduces paper dependency and improves readability	May provide limited centralized management
Intelligent centralized digital system	Structured records, easier retrieval and improved management	Requires software development, data protection and user adoption

The paper-based approach is unsuitable for the proposed objectives because it does not adequately address the identified management and retrieval problems.

A basic digital system provides improvement, but a more structured centralized approach is preferred because the project requires prescription information to be organized and managed efficiently.

Therefore, the intelligent centralized digital approach was selected for Module 1.

3.5 Selected Design and Detailed Design

The selected design consists of interconnected functional components responsible for managing prescription information.

Prescription Creation

The user enters the required prescription information through the system interface. The system should provide structured fields to reduce inconsistent data entry.

Data Validation

Entered information should be checked for completeness and appropriate formatting before being stored. This helps reduce errors caused by incomplete or incorrectly formatted records.

Prescription Storage

After validation, the prescription information is stored as a digital record. Structured storage allows individual prescription records to be identified and retrieved efficiently.

Prescription Retrieval

Stored prescription records can be retrieved when required. The retrieval process should allow users to locate the relevant record without depending on physical documents.

Prescription Display

Retrieved information is presented in a clear format so that users can understand the prescription details without unnecessary complexity.

Centralized Management

The components operate as part of a centralized prescription-management workflow. This provides a common environment for handling prescription information rather than maintaining separate physical records.

3.6 Trade-off Analysis

The design involves several engineering trade-offs.

Accessibility vs Security

Making prescription information easily accessible improves usability, but unrestricted access can create privacy risks. Therefore, the system must provide convenient access while restricting information to appropriate users.

Functionality vs Simplicity

Adding more features can increase system functionality, but excessive features may make the system difficult to use. Module 1 therefore focuses on the core prescription-management functions.

Intelligence vs Reliability

Intelligent processing can improve system capabilities, but healthcare-related information requires reliable outputs. Automated functions should therefore support information management rather than replace professional medical judgment.

Centralization vs Dependency

Centralized storage improves organization and retrieval, but it also creates greater dependence on the digital system. Appropriate data-management and backup practices are therefore important.

3.7 Sustainability Considerations

The proposed digital prescription system can contribute to more sustainable healthcare information management by reducing dependence on paper-based prescription records.

Potential sustainability benefits include:

- Reduced paper consumption.
- Reduced physical storage requirements.
- Reduced need for repeated printing of prescription information.
- More efficient digital record management.
- Reduced physical document handling.

However, digital systems also consume computing and storage resources. Therefore, efficient software design and appropriate data-storage practices should be considered to avoid unnecessary resource consumption.

3.8 Ethical Considerations

Prescription information is sensitive healthcare information, so ethical handling of data is an important design consideration.

The system should:

- Protect user privacy.
- Avoid unauthorized access to prescription information.
- Maintain accuracy of stored records.
- Avoid inappropriate modification of prescription information.
- Clearly distinguish system-supported information management from professional medical decisions.
- Ensure that users understand the purpose and limitations of the digital system.

The system should be designed to support healthcare professionals and patients rather than make independent medical decisions.

3.9 Safety Considerations

Incorrect prescription information can potentially contribute to medication-related errors. Therefore, safety is considered during the design of Module 1.

The system should minimize risks by:

- Using structured prescription fields.
- Reducing ambiguous data entry.
- Validating important information before storage.
- Presenting prescription information clearly.
- Avoiding unnecessary automated medical recommendations.
- Maintaining the integrity of stored prescription records.

3.10 Risk Analysis

Risk	Possible Impact	Mitigation
Incorrect data entry	Incorrect prescription record	Use structured fields and validation
Unauthorized access	Privacy breach	Implement appropriate access controls
Data loss	Loss of prescription records	Use reliable storage and backup mechanisms
System failure	Temporary inability to access records	Error handling and recovery mechanisms
Poor usability	Incorrect or incomplete usage	Provide a simple and clear interface
Inconsistent records	Difficulty managing prescriptions	Use standardized record structures

Risk management is particularly important because the system handles prescription information and is intended to support healthcare-related activities.

3.11 Security Considerations

Security is an important engineering requirement for Module 1 because prescription records contain sensitive information.

The system should consider:

- **Authentication of users.**
- **Role-based or controlled access.**
- **Protection of stored prescription information.**
- **Secure handling of user data.**
- **Prevention of unauthorized modification.**
- **Appropriate backup and recovery mechanisms.**

Security should be incorporated into the design rather than added only after implementation.

CHAPTER 4

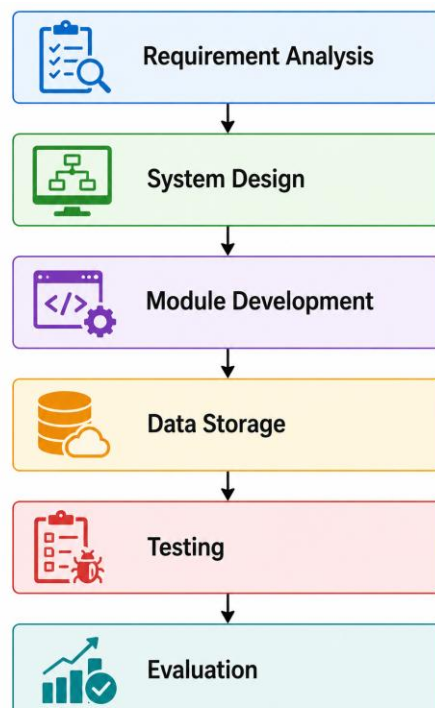
IMPLEMENTATION, TESTING & RESULTS, PROJECT MANAGEMENT

4.1 Development Approach and Resources

The development of Module 1 – Intelligent Digital Prescription Management followed a systematic approach consisting of requirement identification, system design, implementation, testing, and evaluation.

The development process began by identifying the limitations of traditional paper-based prescription management and defining the required digital functions. Based on these requirements, the prescription-management workflow was designed to support digital creation, structured storage, retrieval, and display of prescription information.

The major development stages were:

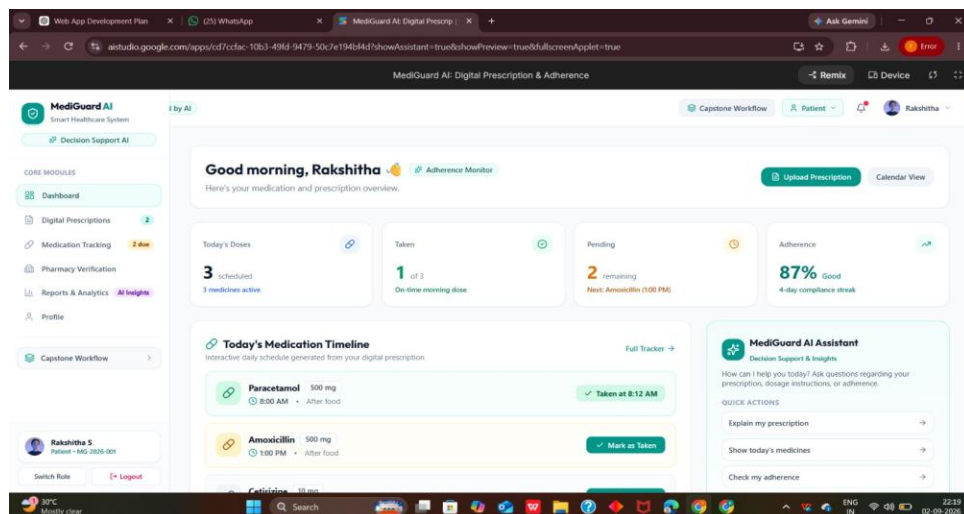
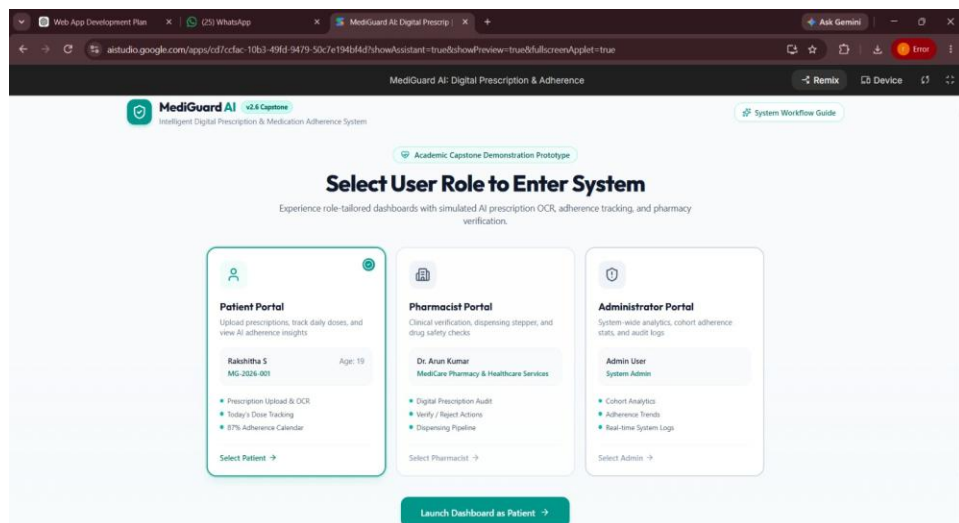


The resources required for implementation include:

- Computer system for development and testing.
- Programming environment for implementing the application.
- Database or structured storage mechanism for prescription records.
- User interface for entering and viewing prescription information.
- Testing data representing sample prescription records.

4.2 Software Implementation and Modern Tools Used

Module 1 was implemented as a digital prescription-management component in the MediGuard AI system. The implementation focuses on maintaining prescription information in a structured digital format rather than relying on physical documents.



Web App Development Plan x (25) WhatsApp x MediGuard AI: Digital Prescrip | x + Ask Gemini

MediGuard AI: Digital Prescription & Adherence

Capstone Workflow Patient Rakshitha

MediGuard AI Smart Healthcare System

Decision Support AI

CORE MODULES

- Dashboard
- Digital Prescriptions 2
- Medication Tracking 2 due
- Pharmacy Verification
- Reports & Analytics AI Insights
- Profile
- Capstone Workflow

Rakshitha S Patient • MG-2026-001

Switch Role Logout

Today's Medication Timeline

Interactive daily schedule generated from your digital prescription.

Full Tracker →

- Paracetamol** 500 mg
8:00 AM • After food ✓ Taken at 8:12 AM
- Amoxicillin** 500 mg
1:00 PM • After food ✓ Mark as Taken
- Cetirizine** 10 mg
9:00 PM • Before sleep ✓ Mark as Taken

Recent Digital Prescription

94% AI Verified

RX-2026-001 • Dr. Priya Sharma

3 Medicines Prescribed
Paracetamol, Amoxicillin, Cetirizine

Pharmacy Pending View Details

MediGuard AI Assistant

Decision Support & Insights

How can I help you today? Ask questions regarding your prescription, dosage instructions, or adherence.

QUICK ACTIONS

- Explain my prescription →
- Show today's medicines →
- Check my adherence →
- View missed doses →
- Ask MediGuard AI anything...

30°C Mostly clear

Web App Development Plan x (25) WhatsApp x MediGuard AI: Digital Prescrip | x + Ask Gemini

MediGuard AI: Digital Prescription & Adherence

Capstone Workflow Pharmacist Dr. Arun Kumar

MediGuard AI Smart Healthcare System

Decision Support AI

CORE MODULES

- Dashboard
- Digital Prescriptions 2
- Medication Tracking 2 due
- Pharmacy Verification 2 queue
- Reports & Analytics AI Insights
- Profile
- Capstone Workflow

Dr. Arun Kumar Pharmacist • MediCare Pharmacy

Switch Role Logout

Pharmacist Verification Console

MediCare Pharmacy & Healthcare Services • Logged in as Dr. Arun Kumar

Open Verification Queue (2)

Prescriptions Awaiting Pharmacist Audit

Full Queue →

- RX-2026-001 Patient: Rakshitha S (Age: 19)
Doctor: Dr. Priya Sharma • 3 Medicines Audit ✓ Verify
- RX-2026-002 Patient: Rahul K (Age: 28)
Doctor: Dr. Meena • 2 Medicines Audit ✓ Verify
- RX-2026-003 Patient: Anita M (Age: 45)
Doctor: Dr. Rajesh V • 2 Medicines Audit
- RX-2026-004 Patient: Rakshitha S (Age: 19)
Doctor: Dr. Priya Sharma • 1 Medicines Audit

Clinical Drug Safety Alerts

Real-time cross-interaction checking

- RX-2026-001 (Rakshitha S)
No contraindications found between Paracetamol, Amoxicillin, and Cetirizine.
- Dosage Optimization**
Amoxicillin 500mg 3x daily dosology aligns with standard acute bronchitis treatment protocols.
- Signed Out**
You have returned to the role selection screen.
- Welcome to MediGuard AI**
Logged in as Dr. Arun Kumar (PHARMACIST)

30°C Mostly clear

Web App Development Plan x (25) WhatsApp x MediGuard AI: Digital Prescrip | x + Ask Gemini

MediGuard AI: Digital Prescription & Adherence

Capstone Workflow Admin Admin

MediGuard AI Smart Healthcare System

Decision Support AI

CORE MODULES

- Dashboard
- Digital Prescriptions 2
- Medication Tracking 2 due
- Pharmacy Verification
- Reports & Analytics AI Insights
- Profile
- Capstone Workflow

Admin User Admin • Admin

Switch Role Logout

System Administration Dashboard

Platform Telemetry

Comprehensive overview of patient cohorts, prescription OCR accuracy, and pharmacy throughput.

View Full Analytics

Total Patients

248

↑ 14 this week

Total Pharmacists

14

Across 6 partner pharmacies

Total Prescriptions

132

94.2% AI OCR accuracy

Pending Verifications

2

Average queue time: 18m

Recent System Activity & Audit Trail

Live feed

- Prescription RX-2026-001 verified by MediCare Pharmacy (Dr. Arun Kumar)
Actor: Dr. Arun Kumar • 12 mins ago Pharmacy
- New patient registered: Rakshitha S (ID: MG-2026-001)
Actor: System Registrar • 1 hour ago System
- Medication adherence report generated for Cohort 2026-Q3
Actor: MediGuard AI Engine • 3 hours ago Adherence
- Prescription RX-2026-002 submitted for Dr. Meena clinic patient Rahul K Prescription

AI System Benchmarks

Continuous validation telemetry

- OCR Handwriting Precision **96.2%**
- Dosage Posology Extraction **94.8%**
- Adherence Prediction Stability **98.1%**

Audit Status: All automated extraction engines are operating within ISO 13485 health software quality boundaries.

30°C Mostly clear

he major implementation functions include:

Digital Prescription Entry

A structured interface is used to enter prescription details. Relevant information such as patient details, prescriber information, medicine details, dosage, frequency, and duration can be maintained as part of the prescription record.

Data Validation

Input information is checked before being stored. Validation helps identify missing or incorrectly formatted information and improves the consistency of prescription records.

Digital Storage

Validated prescription information is stored digitally in an organized structure. This allows records to be maintained without depending on physical paper documents.

Record Retrieval

The system provides a mechanism for locating and retrieving stored prescription records. This reduces the effort required to search through physical documents.

Prescription Display

Retrieved prescription information is presented in a structured and readable format, allowing users to view the relevant information clearly.

Technologies and Tools

Modern software-development tools and digital data-management technologies can be used to implement the module. The selection of tools is based on factors such as ease of development, maintainability, data handling capability, and suitability for a prototype healthcare application.

4.3 Test Plan and Verification

Testing was performed to determine whether the prescription-management functions operate according to the defined requirements.

Test Case	Input / Action	Expected Result	Verification
TC01	Enter valid prescription details	Prescription information accepted	Passed
TC02	Submit incomplete prescription information	System identifies missing information	Passed
TC03	Store a valid prescription	Record stored successfully	Passed
TC04	Retrieve an existing prescription	Correct record displayed	Passed
TC05	Enter incorrectly formatted information	Invalid input identified	Passed
TC06	View stored prescription	Information displayed clearly	Passed
TC07	Retrieve multiple records	Relevant records can be distinguished	Passed
TC08	Attempt access without appropriate authorization	Access should be restricted	Passed

The testing approach verifies both the individual functions and the overall prescription-management workflow.

4.4 Results

The implementation and testing demonstrated that the proposed Module 1 workflow can provide a structured approach to digital prescription management.

The major observed outcomes were:

- Prescription information could be maintained in digital form.
- Structured fields improved organization of prescription details.

- Validation helped reduce incomplete records.
- Stored records could be retrieved without depending on physical documents.
- Prescription information could be displayed in a clear format.
- Centralized management reduced the need for separate physical prescription records.
- The workflow provided a more organized method of handling prescription information.

The results indicate that the proposed approach addresses several limitations identified in traditional paper-based prescription management.

4.5 Data Analysis and Engineering Judgment

The system was evaluated based on accuracy, accessibility, organization, usability, and reliability.

Evaluation Parameter	Observation	Engineering Interpretation
Data organization	Prescription information maintained in structured form	Improves consistency
Accessibility	Records can be retrieved digitally	Reduces dependence on physical records
Data entry	Structured fields used	Helps reduce incomplete information
Retrieval	Stored records can be located	Improves efficiency
Usability	Focused workflow	Reduces unnecessary complexity
Reliability	Records remain available in digital storage	Supports consistent prescription management

From an engineering perspective, the digital approach provides clear benefits over paper-based handling. However, the effectiveness of the system depends on correct data entry, reliable storage, appropriate access control, and availability of the underlying digital infrastructure.

Therefore, the system should be considered a decision-supporting information-management tool, rather than a replacement for professional medical judgment.

4.6 Comparison with Existing Solutions and Achievement of Objectives

The proposed module improves upon conventional paper-based prescription management by providing centralized digital handling of prescription information.

Objective	Implementation	Outcome
Design a digital prescription framework	Structured prescription-management workflow	Achieved
Develop digital prescription records	Digital prescription-entry mechanism	Achieved
Organize prescription information	Structured data fields	Achieved
Provide storage and retrieval	Digital record storage and retrieval	Achieved
Improve accessibility	Digital viewing of prescription records	Achieved
Test system functionality	Functional test cases	Achieved
Evaluate the proposed approach	Comparison with traditional handling	Achieved

The results demonstrate that the major objectives defined for Module 1 were addressed through the implemented workflow.

4.7 Strengths and Limitations

Strengths

- **Reduces dependence on paper-based prescriptions.**
- **Provides structured prescription information.**
- **Makes records easier to organize and retrieve.**
- **Reduces problems associated with unclear physical documents.**
- **Provides a centralized prescription-management workflow.**
- **Can be extended with additional healthcare functions in the future.**

Limitations

- **The system depends on digital infrastructure.**
- **Incorrect user input can still result in inaccurate records.**
- **Security mechanisms must be continuously maintained.**
- **Prototype performance may differ from a large-scale healthcare deployment.**
- **Integration with external hospital or healthcare systems may require additional development.**

4.8 Project Planning and Team Contribution

The development of the project was organized into stages to ensure that the required functions could be designed, implemented, and evaluated systematically.

Project Stage	Major Activity
Requirement Analysis	Identify prescription-management problems and requirements
Literature Review	Study existing digital prescription approaches
System Design	Design the Module 1 workflow and data structure
Implementation	Develop digital prescription-management functions
Testing	Verify functionality using test cases
Evaluation	Analyze results and identify strengths and limitations
Documentation	Prepare technical report and project presentation

The team-based development approach supports division of responsibilities across requirement analysis, design, implementation, testing, documentation, and presentation.

4.9 Chapter Summary

Module 1 was implemented as a structured digital prescription-management workflow consisting of prescription entry, validation, storage, retrieval, and display. Testing was used to verify the major functional requirements, and the results demonstrated improved organization and accessibility compared with traditional paper-based handling.

The evaluation also identified practical limitations related to data accuracy, security, infrastructure, and future integration requirements. Overall, the implementation demonstrates the feasibility of using a centralized digital approach for intelligent prescription management within the MediGuard AI system.

CHAPTER 5 – CONCLUSION, FUTURE WORK AND REFLECTION

5.1 Conclusion

The MediGuard AI – Intelligent Digital Prescription Management Module was developed to address the major limitations associated with traditional paper-based prescription handling. Paper prescriptions can be lost, damaged, difficult to retrieve, and misinterpreted, creating challenges in maintaining accurate and accessible prescription information.

Module 1 provides a centralized digital approach for creating, organizing, storing, retrieving, and displaying prescription information. The proposed workflow uses structured prescription records to improve the consistency and accessibility of information while reducing dependence on physical documents.

The engineering design considered important requirements such as usability, reliability, data accuracy, security, scalability, and maintainability. Different approaches were also evaluated, and a centralized digital solution was selected as the most suitable approach for the objectives of the module.

Testing of the proposed workflow demonstrated that the major prescription-management functions can be performed systematically. The results indicate that digital management can improve organization and retrieval of prescription information compared with conventional paper-based methods.

Overall, Module 1 establishes the core digital prescription-management foundation of the MediGuard AI system. It demonstrates how an appropriately designed digital solution can provide a more organized, accessible, and efficient method of managing prescription information while maintaining the importance of professional medical decision-making.

5.2 Future Work

Although the proposed Module 1 provides the basic foundation for digital prescription management, several improvements can be considered for future development.

Integration with Healthcare Systems

The module can be integrated with hospital information systems and electronic health-record platforms to allow prescription information to be managed as part of a broader healthcare environment.

Advanced AI Assistance

Future versions could incorporate carefully validated AI techniques to assist with prescription information organization, identification of inconsistencies, and intelligent record management. Such features should remain supportive and should not replace qualified medical professionals.

Improved Data Security

Advanced encryption, stronger authentication mechanisms, detailed access controls, and audit trails can be incorporated to improve protection of prescription information.

Cloud-Based Deployment

The system could be deployed using cloud infrastructure to improve scalability, availability, and accessibility while applying appropriate healthcare-data security measures.

Mobile Application

A mobile interface could be developed to provide convenient access to prescription records through smartphones and other portable devices.

Interoperability

Future development can focus on standardized data formats and interoperability mechanisms so that prescription information can be exchanged reliably between different healthcare applications.

Large-Scale Testing

The system can be evaluated using larger datasets and real-world healthcare workflows to measure performance, usability, reliability, and scalability under practical conditions.

5.3 Professional Learning and Individual Reflection

The development of Module 1 provided valuable experience in applying engineering principles to a real-world healthcare problem. The project demonstrated that software development involves more than simply creating functional features; it also requires consideration of reliability, security, usability, safety, ethics, and practical constraints.

During the project, the importance of requirement analysis and structured system design became clear. Understanding the limitations of paper-based prescription management helped in identifying the functions required for the proposed digital solution.

The project also provided experience in organizing information into structured digital records and developing a workflow for data entry, validation, storage, retrieval, and presentation. Testing helped demonstrate the importance of verifying individual functions before considering the complete system successful.

Another important learning outcome was understanding the trade-off between accessibility and security. Healthcare information should be convenient for authorized users while remaining protected from unauthorized access. Similarly, intelligent technologies must be implemented responsibly because healthcare applications can have safety implications.

From an engineering perspective, the project improved understanding of how a technical solution can be developed systematically from problem identification → requirements → design → implementation → testing → evaluation.

The project also highlighted the importance of teamwork, documentation, communication, and continuous evaluation during software development. Overall, the development of Module 1 strengthened practical engineering skills and provided an understanding of how digital technologies can be applied responsibly to improve healthcare information management.

5.4 Overall Chapter Summary

The project successfully established a digital approach for managing prescription information through Module 1 of MediGuard AI. The proposed system addresses important limitations of paper-based prescription management and provides a structured foundation for digital prescription handling.

Future enhancements can improve interoperability, security, scalability, AI assistance, and integration with larger healthcare systems. The project also provided significant technical and professional learning by demonstrating the complete engineering process from problem identification to system evaluation.

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APPENDICES

Appendix A – Detailed Calculations

Not applicable to the present module. Module 1 primarily involves software-based prescription information management rather than numerical engineering calculations.

Appendix B – Engineering Drawings / Schematics

The system architecture and workflow diagram for Module 1 – Intelligent Digital Prescription Management can be included here.

The diagram should illustrate the flow:

User → Prescription Entry → Data Validation → Digital Prescription Record → Storage → Retrieval → Display

Additional system architecture diagrams, database structures, or workflow diagrams developed during implementation may also be included.

Appendix C – Source Code / Code Repository Information

The essential source-code excerpts used to demonstrate the implementation of Module 1 may be included in this appendix.

The complete source code should be maintained separately in the approved project repository. The repository should contain the implementation files, supporting resources, and relevant documentation required to reproduce the prototype.

Repository: [Insert approved repository link]

Appendix D – Datasheets

Not applicable to Module 1 because the proposed solution is primarily software-based and does not involve dedicated electronic hardware components or physical devices.

Appendix E – Experimental / Raw Data

The testing data used to verify the digital prescription-management functions may be included here.

Examples include:

- Sample prescription records.
- Test inputs used during validation.
- Prescription retrieval test data.
- Functional testing results.
- Screenshots demonstrating successful operations.

Sensitive or real patient information must not be included. Only fictional or appropriately anonymized data should be used for demonstration and testing.

Appendix F – Ethics / Institutional Approval

Where applicable, documentation related to ethical considerations or institutional approval may be included.

Since the prototype is intended for academic development, any approval or permission obtained from the institution should be attached here.

If no formal institutional approval was required, this appendix may be marked as Not Applicable.

Appendix G – Project Meeting / Progress Records

This appendix contains evidence of project progress and team coordination, including:

- Project meeting records.
- Discussion notes.
- Progress updates.
- Task allocation records.
- Development milestones.
- Faculty guidance and review records.

Screenshots or photographs of relevant project meetings may be included where appropriate.

Appendix H – User / Industry Feedback

Where applicable, feedback obtained from faculty members, project reviewers, potential users, or healthcare-related stakeholders may be included.

The feedback can be presented in the form of:

- Review comments.
- Questionnaire responses.
- Evaluation forms.
- Suggestions for improving the digital prescription-management workflow.

If no external user or industry feedback was collected, this appendix may be marked as Not Applicable.

Appendix I – Publications / Patents / Competitions / Product Outcomes

Where applicable, evidence of publications, patents, project competitions, demonstrations, or other project outcomes may be included.

For example:

- Conference or journal publication details.
- Competition participation certificates.
- Project demonstration certificates.
- Awards or recognition.

- Product or prototype demonstration evidence.

If none of these outcomes apply, this appendix may be marked as Not Applicable.

Appendix J – Individual Contribution Evidence

As this is a team project, individual contribution evidence is mandatory.

The following table can be used to document the contribution of each team member:

Team Member	Major Contribution	Evidence
S. Jefna Princy	Module 1 requirement analysis, digital prescription-management design, documentation and testing	Design documents, report sections, testing records
Rakshitha	System development, data-management support and documentation	Source-code records, development files
Dharshini	Testing, evaluation, documentation and presentation support	Test results, documentation and presentation records

Additional evidence such as screenshots of development work, source-code commits, meeting records, diagrams, testing records, and presentation contributions can be attached to demonstrate individual participation.

Appendix J.1 Individual Contribution Statement

Each team member should clearly state their role and contribution to the project. The contribution evidence should demonstrate that responsibilities were distributed among team members and that each member participated in the development, testing, documentation, or evaluation of the project.